

Oscar Yang

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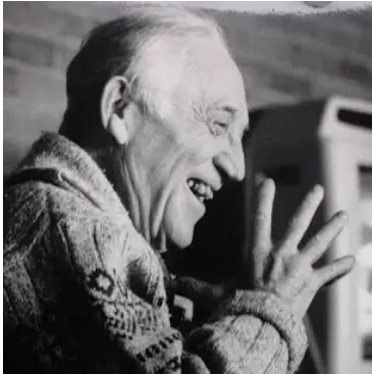
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How much should we value a dying man?

Does the value of a terminally ill person outweigh the costs of providing care and the potential suffering of intensive care? Morrie Schwartz, a sociology professor at Brandeis University and the main subject of the best-selling book *Tuesdays with Morrie*, believed it did. He wasn't scared of death, or aging. Rather, he invited it. In his final year, he suffered from ALS and was bedridden for most of the day. However, throughout his time, he challenged the societal norms of death being melodramatic and taboo. In an ABC interview, he said, "This culture is so stuck on death, in terms of its fear, hiding it, not knowing what to do with it, that what I'm saying is, is there an alternative way of looking at it?" He beckoned us to not be afraid of death, to accept that it eventually comes to everyone. Schwartz invited others to embrace death as an inevitable part of life, suggesting that openness and acceptance can bring meaning. In that case, if death is unavoidable, why do we focus on prolonging unnecessary suffering, rather than alleviating it?

Morrie Schwartz's reflections on life and death give a compelling case for why physician-assisted suicide (PAS) should be an option for terminally ill individuals. Schwartz, who faced the debilitating effects of ALS, openly shared the emotional and physical toll of his illness, challenging societal fears about death. His experiences emphasize the extreme agony that many patients facing terminal illness go through as they lose their autonomy, self-respect, and



sense of control over their lives. By giving people the freedom to choose how and when they pass away, PAS offers a humane substitute that restores their feeling of autonomy and ends needless suffering. For people who are experiencing excruciating agony or humiliation, PAS is a brave choice to meet death on their own terms rather than a sign of hopelessness.

Although PAS's ethical implications are frequently questioned, strong protections in states where it is permitted guarantee that it is a free and informed decision. In addition to alleviating physical suffering, PAS helps patients cope with the psychosocial effects of terminal disease and gives them the assurance that comes from being in charge of their last hours. Schwartz's candor regarding death serves as a reminder that while dying might have purpose and dignity, many people believe that dignity necessitates the ability to avoid excruciating pain. In addition to upholding individual liberty, legalizing PAS encourages a more humane view of death, one that values the welfare of those who must deal with it and accepts it as a normal aspect of life.

PAS is an inherently moral issue focused on respect for autonomy and the alleviation of suffering, mirroring philosophical discussions on life's subjective value. Ognjen Arandjelović denies the existence of life value, stating, "the claim of an absolute value of life, or indeed anything at all, is nonsensical, a *contradictio in adjecto*, for the very meaning of the word 'value' is comparative in nature." (Arandjelović) This perspective emphasizes that individuals are best placed to ascertain their personal quality of life and the stage at which this suffering becomes unbearable and outweighs the benefits of continued existence. Arandjelović further continues to argue that subjective value emanates from personal experiences, pegged on one's capacity to either experience pleasure or endure suffering: "A decision on whether to continue living or to

commit suicide is predicated on one's prediction of possible future lives" (Arandjelović). In this context, PAS is a moral response to unbearable suffering that asserts the individual's right to make decisions in their life with dignity.

Friedrich Nietzsche gives a philosophical basis for supporting PAS within the context of retrieving autonomy. Nietzsche rejects both pessimistic resignation and blind optimism, arguing instead for a nuanced understanding of suffering and agency. Nietzsche, in his discussion on the value of life, denies traditional morality, "to understand life historically, teleologically, as the unfolding of a state, a nation, a species, is to misconstrue the lesson of existence: in becoming, there is no meaning." This denial of meaning imposed upon him also goes along with the notion that individuals must create their own values, including how they approach death. In this light, PAS turns into a liberating act of affirmation of the "will to power" against the body with all one's weaknesses. Nietzsche's philosophy urges society toward a caring but flexible approach that respects personal dignity while stimulating open debate about the subjective value of life and death.

On the other hand, the idea of ending a life, or even helping someone end their life is morally complicated. "‘How much will my loved ones suffer?’ one cannot help but ask him/herself." (Arandjelović) Besides the terminally ill patient, there's a multitude of other lives affected by this decision. Arandjelović highlights the question perfectly, asking whether this final decision is selfish. The moral argument would be "to exercise this legal right would be morally impermissible for it would cause undue harm to others." (Arandjelović) The process of death is slow and creeps upon a person. To end one's own life early would be to harm others in an attempt to save oneself. Some put it that "suicide is the greatest piece of cowardice," (Schopenhauer) but what could be more unassailable than one's life? The decision is neither

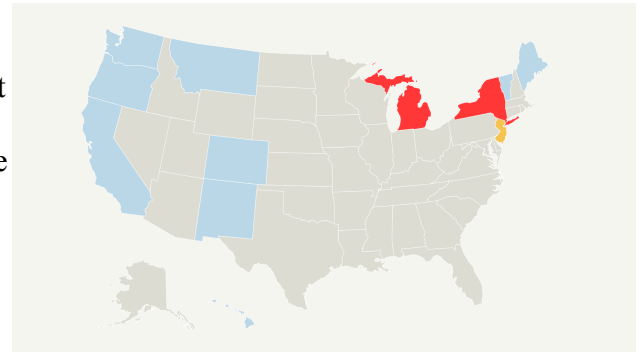
inherently evil nor unjust. As a medical professional, there is a dilemma of whether a patient's suffering and inevitable death can justify an early exit. Even if the decision is selfish and cowardly, we should be able to allow a person to go out on their terms, and be able to touch, hear, and see their families in their last moments.

Scientific research on physician-assisted suicide shows how complex it is to balance moral issues against the need for care with transparency and compassion. In countries like Belgium and the Netherlands, PAS is considered a legal option under specific circumstances, especially for patients with terminal illnesses. As noted, PAS is an option for "serious irreversible diseases, such as cancer, that are accompanied by unbearable physical suffering and that finally lead to death" (Deschepper). However, the application becomes far more complex when mental suffering is the primary justification. "Unbearable mental suffering due to an irreversible disease" according to Deschepper, for example, accounted for 3.5% of reported euthanasia cases in 2011. Here, strict guidelines entailing advice from at least two other physicians were put in place. These reflect the need to ensure such demands are free from fraud and continuous, without abuses of the law and misdiagnosis, on the one hand.

Physicians, however, face immense challenges in handling PAS requests, especially in cases of mental suffering. Without "objectively measurable symptoms," (Deschepper) doctors must rely heavily on the patient's subjective descriptions of their suffering, creating uncertainty about whether the request stems from "a genuine and constant form of unbearable suffering" or from treatable conditions like severe depression. Also, patients who perceive that their wishes will be refused may resort to desperate measures, such as suicide threats, to make physicians comply with their demands. To deal with these moral and emotional stresses, professionals encourage multidisciplinary group decisions to spread the onus, as this approach will ensure

"decisions are made in a transparent way" (Deschepper) while giving the caregivers emotional and professional support. Such teams help build a system where the rights and needs of patients and physicians are respected, reinforcing ethical integrity in end-of-life care.

The political framework concerning physician-assisted suicide (PAS) reflects significant differences in policy, public opinion, and legislative action across the country. In the United States,



Blue: legalized, Red: considered, Yellow: amended, Gray: illegal

"Death with Dignity" laws provide an opportunity for patients with terminal illnesses to make decisions about how and when they die. This "allows certain people with terminal illness to voluntarily and legally request and receive a prescription medication from their physician to hasten their death." (Death With Dignity) So far, only a handful of states, particularly Oregon, Washington, and California, have passed these restrictions, requiring strict stipulations such as verification of terminal illness by two physicians and that the patient has to self-administer the medication. The highly controlled process shows how states try to balance individual autonomy with societal protection. Advocates argue that would allow qualifying individuals a "peaceful, humane, and dignified" option at the end of life, which at the same time should prevent possible abuses of the law. Yet, some faith-based healthcare institutions and professionals introduce continuing ethical and political barriers to extending PAS laws throughout the United States.

Opposition to PAS arises from more sweeping political and cultural disputes over the scope of government in controlling life-and-death decisions. Critics protest that these acts undermine the sanctity of life and could lead to coercion or devaluation towards individuals with disabilities or chronic ill health. Those concerns reflect the tension between the personal desire

for autonomy, on the one hand, and, on the other, the necessity for protection to society. Still, proponents stress that PAS laws are voluntary, with no mandatory participation required of either physicians or patients. PAS laws become “an extension of individuals' right to control the most personal aspects of their own existence, the option of dying with human assistance.” (Worsnop) This reflects a political compromise in permitting PAS to be cast not as a universal right, but as a personal choice subject to tight regulations.

The debate on PAS encompasses a complicated field of moral, philosophical, and political views about end-of-life care. Advocates argue that PAS allows one to exercise autonomy, lessen suffering, and die with dignity. However, critics point out moral dilemmas, risks to society, and the emotional burden that such decisions can inflict. Philosophers such as Morrie Schwartz, Friedrich Nietzsche, and Ognjen Arandjelović help to place greater emphasis on subjective experience, self-determination, and autonomy on the two sides of the debate: that is, life and death and how they are valued. The movement for "Death with Dignity" laws is within a cautious but forward-thinking framework that balances the rights of the individual against protections against misuse. As society continues to address the concepts of terminal illness and the inevitability of death, the controversy surrounding PAS would challenge society to develop a compassionate, nuanced, and inclusive stance about human dignity and its choices.

Reflection

I enjoyed writing and researching about physician-assisted suicide. My main inspiration for this was Morrie Schwartz, but more specifically, his many lessons in *Tuesdays with Morrie*. The topic of physician-assisted suicide came up while thinking about how he valued his end-of-life experience. From there, I explored the different perspectives on this issue, including philosophical, moral, and political views. While researching, I found Nietzsche and Arandjelović, who added the philosophical aspects to my writing. For textual analysis, I relied on Morrie Schwartz's reflections in *Tuesdays with Morrie* to provide a personal and emotional exploration of the question. I also included empirical data and policy frameworks from contemporary sources. Death with Dignity, the organization, answers a lot of the legal questions about the permissibility of PAS. These practical means showcased how PAS is implemented and regulated in different contexts.

The topic of physician-assisted suicide is personal, ethically complex, and culturally complicated, which made defining a clear purpose and audience important to my composition. The intended audience was the general public, including individuals who might be deliberating with the philosophical and practical questions surrounding end-of-life care. I organized my writing to begin with Morrie Schwartz and his influence, and then transfer to the moral and philosophical aspects of the topic. Then, I included the scientific problems and legal prospects to top it off and present the facts and issues to the public consensus. I included pictures and videos of my main subjects, Morrie Schwartz and Deb Robertson, to help appeal to the emotions of my readers. I also imputed graphs to show the legal layout of the US in terms of PAS laws to show the readers how little opportunity there is to access this type of care. Working across these modes allowed me to constantly rethink the effectiveness of my arguments and evidence. Eventually,

this method allowed me to address my purpose, encouraging a compassionate, nuanced view of death, while respecting the diverse perspectives of my audience.

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